# Complete Case Report — TKT-00517

**Submission:** all five components, case summary, and four redacted warm-transfer records in one PDF. **Evidence boundary:** supplied mock ticket only; no live tenant access or production actions claimed.

| Metadata | Entry |
|---|---|
| Ticket / intake | TKT-00517 \| 09/03/2026 08:44 ET \| assigned Tier 1 Healthcare IT Support |
| Learner | Avery Deng Jan |
| Report preparation timestamp | 11/11/2026 14:00 CST (15:00 ET); report-drafting time, not incident action time |
| SOP / reference baseline | FLR-01 v1.0; Basic Cloud Security Check Procedure v1.0; PHI Redaction Procedure and Verification Checklist v1.0; HIPAA SOPs and Pre-Approved Runbooks v1.0 (effective 09/04/2026); EVID-01, IR-01, PRIV-01, PRIV-02, M365-01; Escalation Team Reference v1.0; remaining supplied training references |
| Redacted exhibits | EX-01 ticket/reporter digest; EX-02 message digest; EX-03 Entra events; EX-04 account posture; EX-05 SharePoint configuration and Purview activity; WT-SOC, WT-IAM, WT-IG, WT-PRIV. Originals remain in supplied restricted training source packet; no actual repository deposit verified. |

**Timing caveat:** HIPAA training runbook baseline states effective 09/04/2026, after the synthetic events on 09/03/2026. It is applied here as the assignment's designated training baseline, not represented as a policy confirmed effective at the event time.

## 1. Component 1 — Cybersecurity Containment

| Required field | Completed assessment / record |
|---|---|
| Threat category | Primary: **phishing / social engineering**. Subsequent suspected credential compromise/account takeover; possible unauthorized access/data exposure is a question for specialists (Four Cybersecurity Threat Categories, multi-stage guidance). |
| Signal 1 — EX-01 / EX-02 | 09/03/2026 08:14 ET purported support message used urgency and a deceptive verification link; Attending Physician reports selecting link and entering work credentials at approximately 08:17 ET. (Ticket Evidence A; reporter statement.) |
| Signal 2 — EX-03 | Entra: 08:11 ET Boston success; 08:29 ET Phoenix success with unfamiliar sign-in properties and unavailable device ID; 08:36 ET Phoenix failure 50053 with high-confidence risk block. Reporter states in Boston without travel. Anomalous signal ≠ confirmed attacker. (Ticket Evidence B.) |
| Step 1: Isolate / contain | **Documented response:** open incident under TKT-00517 (ticket opened 08:44 ET by assigned queue); instruct account holder not to revisit link, re-enter credentials, or approve unexpected prompts. **Status:** user instruction and SOC/IAM activation prepared in report; no evidence these were communicated or enacted. Any password reset, session revocation, or account disablement is **pending authorized SOC/IAM action** following care-continuity review; Tier 1 did not execute it. (FLR-01 Step 1; IR-01.B.) |
| Step 2: Preserve evidence | EX-01 to EX-05 indexed from unchanged synthetic source packet: reporter statement, message ID, Entra log, posture and file/audit extracts; original source retained unchanged as supplied, redacted descriptions only in report. No deletion, forward, investigation, file edits, or unapproved export. **Actual restricted repository ingest/integrity hash:** not verifiable in exercise. (FLR-01 Step 2; EVID-01.) |
| Step 3: Escalate to SOC | WT-SOC below, drafted 11/11/2026 14:00 CST for Contoso Health Security Operations Center via restricted ServiceNow security/privacy incident task. **Dispatch/acknowledgment pending**; not falsely represented as received. (FLR-01 Step 3; IR-01.B.) |
| Unresolved questions | Who controlled the Phoenix sign-in/session? What sessions remain active? What response action is authorized after clinical-continuity review? Did the recorded file activity involve an unauthorized actor? |

## 2. Component 2 — Cloud Security Verification

Pre-incident values below distinguish supplied earlier state from any baseline that the packet does not document. No new portal queries or post-remediation readings were performed. (Basic Cloud Security Check Procedure, Checks 1–3.)

| Check | Pre-incident / expected state | Observed state and time | Outcome / routing |
|---|---|---|---|
| Sign-in / policy | 08:11 ET, Boston success from clinical device (EX-03). Intended clinical MFA Conditional Access baseline: CA-CLINICAL-MFA. Historical exception approval and expiry not provided. | 08:29 ET Phoenix success, unfamiliar-properties risk, device ID unavailable; 08:36 ET 50053 risk block (EX-03). CA-CLINICAL-MFA **did not apply**; CA-TEMP-EXCEPTION with blank end date (EX-04). | **Fail** policy drift → IAM; **Review** risky sign-in → SOC. |
| MFA / methods | Authenticator registered in current extract; no historical registration timestamp or prior enforcement evidence supplied (EX-04). | Authenticator registered; no FIDO2 key registered. Clinician did not approve an Authenticator prompt; CA-CLINICAL-MFA did not apply (EX-01, EX-04). | **Review** sign-in MFA enforcement (not proven). IAM assess policy/exception; absent FIDO2 alone is not a failure without a requirement. |
| Audit visibility | Prior availability/retention and Tier 1 role grants not described. | Entra sign-in records and Purview FileAccessed event are supplied for 09/03/2026 08:32 ET (EX-03, EX-05). | **Pass** supplied evidence availability; permission-specific live check not independently testable. **No Purview Audit Administration transfer triggered.** |

**Cloud-security transfer:** WT-IAM (draft 11/11/2026 14:00 CST). Request Conditional Access exception review and authorized account/session remediation. Evidence is historical; no independent post-remediation state available.

## 3. Component 3 — Data Security Audit

| Aspect / control | Evidence / observation | Outcome | Routing and boundary |
|---|---|---|---|
| Storing — approved tenant/library, retention, service encryption | EX-05: SharePoint ED Case Conference / Shared Documents / Week 35 (repository label only; PHI-bearing filename replaced by Artifact A-01); Microsoft 365 encryption at rest active; no exception. Tenant approval, sensitivity label and retention requirements not given. | **Pass** encryption; **Review** unverified tenant / retention baseline. | Information Governance verify baseline; no invented labeling/retention obligation. |
| Sharing — link specificity and external exposure controls | EX-05: Anyone with the link created 08/28/2026; no expiration, no password. Clinical PHI-sharing context; external recipients / actual link uses not established. | **Fail** training governance baseline (Anyone link). | Information Governance: review authorization, link scope and appropriate remediation; Tier 1 makes no change. (M365-01; PRIV-01.B.) |
| Accessing — permissions, guests and audit activity | EX-05: Purview FileAccessed 09/03/2026 08:32 ET attributed to Affected Workforce Account from Phoenix documentation IP. No direct/group/guest/inherited permission list given. | **Review** identity of actor and permissions; event exists, not evidence of human identity. | SOC investigate session; Information Governance review permission history; Privacy Office consider ePHI implications. |

**Governance transfer:** WT-IG drafted. Preserve original link and permission state before authorized modification; do not infer external access, file acquisition, or breach. (Data Security Safeguards Reference; M365-01.)

## 4. Component 4 — PHI Redaction

Applied to the content of this submission, including transfer body text and any document metadata created here. Original evidence files were **not modified**. Substitution standard: patient name → Patient A; MRN → [MRN REMOVED]; birth date → [DOB REMOVED]; provider name → Attending Physician; username/email → Affected Workforce Account; PHI-bearing filename/path → Artifact A-01. No full suspicious URL or sender/email address is reproduced. This is incident-record minimization, **not a formal HIPAA de-identification determination**.

| Artifact | Sensitive types / substitutions | Hidden-data and export review | Result |
|---|---|---|---|
| EX-01 ticket digest | Provider name → Attending Physician; account → Affected Workforce Account; patient name → Patient A; MRN → [MRN REMOVED]; DOB → [DOB REMOVED]. | Rewritten excerpt; document text, headings, tables and metadata reviewed. | Clean |
| EX-02 suspicious message digest | Sender email, message ID, original link/URL path withheld from plain text; evidence ID references retained; no clickable suspicious link. | No original email attachment, screenshot, hyperlinks, alt text or embedded object in PDF. | Clean |
| EX-03 Entra audit digest | Workforce identity replaced; safe documentation-range IPs retained as non-identifier event signals with times; any user principal name omitted. | Tables/text and PDF extraction reviewed. | Clean |
| EX-04 account posture | Provider/workforce identifiers replaced; CA policy IDs retained as non-PHI configuration evidence. | Table contents / PDF extraction reviewed. | Clean |
| EX-05 file + audit digest | Patient-bearing filename and source path → Artifact A-01; workforce identity → Affected Workforce Account; no source path or link target included. | Text, filenames, URL targets, metadata and extract reviewed. | Clean |
| WT-SOC / WT-IAM / WT-IG / WT-PRIV | All subject headings and summary fields use anonymous role labels; no copied identifier or PHI-bearing filename. | Each summary reviewed for text, headers, comments, links, attachments, and metadata; no screenshots embedded. | Clean |
| Final PDF | Document title and creator metadata scrubbed of patient/workforce/source-file identifiers. | PDF reopened and text extraction scanned; no hidden source attachments. | Clean |

**Verification timestamp:** 11/11/2026 14:00 CST report preparation (completion review recorded during artifact assembly). For any original source screenshots/embedded metadata inaccessible in this PDF, original-asset redaction is **not claimed**; restricted originals remain separate. (PHI Redaction Procedure v1.0.)

## 5. Component 5 — HIPAA Follow-and-Escalate

| Required field | Checklist / exact runbook reasoning |
|---|---|
| Checklist review before SOP-fit | HIPAA Compliance Checklist, sections A–D, reviewed in report preparation on 11/11/2026 14:00 CST. A: incident documented; authorized containment not verified complete; SOC handoff draft pending dispatch. B: clinical SharePoint file/ePHI possible, evidence distinguished from actual viewing; minimum-necessary. C: all designated privacy determinations unresolved. D: IDs/times and four handoff records provided; acknowledgment and restricted-record submission pending. |
| SOP-fit criteria | **IR-01.B**: credentials entered and risky sign-in → immediate SOC / IAM. **IR-01.C**: possible ePHI file → PRIV-01 parallel. **PRIV-01.A**: possible ePHI + suspected session misuse / anomalous FileAccessed → Privacy Office. **PRIV-01.B**: Anyone link to clinical PHI → Information Governance + Privacy Office. **EVID-01** and **M365-01** require source preservation. **PRIV-02** reserves breach assessment to Privacy Office. |
| Documented signal chain | EX-01/02: credentials entered ~08:17 ET. EX-03: unfamiliar Phoenix success 08:29 ET / risk block 08:36 ET. EX-04: CA exception blank end date / policy not applied. EX-05: Anyone link created 08/28 and FileAccessed 08:32 ET. Component 4: report exhibits minimized and checked clean. |
| SOP-fit result | **Documented triggers fit approved routing runbooks; decision on possible impermissible use/disclosure and risk assessment requires escalation.** Tier 1 cannot resolve factual ambiguity. Privacy summary WT-PRIV prepared, not transmitted. |
| Unresolved privacy question | Did a person without authorization acquire/view individually identifiable health information, and what actions and notification decisions (if any) follow under Privacy Office review? FileAccessed denotes a service event, not human attribution. |
| Prohibited determinations check | No conclusion made concerning breach, impermissible disclosure/use, actual human access, reportability, notification, or autonomous severity classification. |

## 6. Case Summary

**Incident facts (100–150 words):** On 09/03/2026, an Attending Physician reported receiving a support-impersonation message at 08:14 ET and entering workforce credentials through a suspicious link at approximately 08:17 ET. A supplied Entra extract shows an earlier Boston success at 08:11 ET, a Phoenix success marked unfamiliar at 08:29 ET, and a blocked Phoenix attempt at 08:36 ET. The account posture extract shows that the intended clinical MFA Conditional Access policy did not apply because of an open-ended temporary exception, despite a registered Authenticator method. A clinical SharePoint case artifact had an Anyone link without expiration or password. A Purview FileAccessed event at 08:32 ET referenced the affected account and Phoenix location. The event does not identify a human actor. The supplied record supports specialist review, not any Tier 1 finding that information was actually exposed.

**Containment and evidence status:** Incident opened at 08:44 ET per source packet; FLR-01 steps mapped in sequence. User warning, authorized account/session containment, restricted repository preservation and handoff transmissions are **pending confirmation** rather than represented as completed. Source evidence unchanged; EX-01–EX-05 are redacted digests. Clinical continuity review required before disruptive identity action.

**Escalations and owners:** WT-SOC → Contoso Health Security Operations Center; WT-IAM → Identity & Access Management Operations; WT-IG → Information Governance; WT-PRIV → HIPAA Privacy & Compliance Office. All four are prepared for restricted ServiceNow routing; no receipt or acknowledgment recorded. Purview Audit Administration is not triggered by the supplied available logs.

**Open questions / next checkpoint:** Confirm dispatch and acknowledgment timestamps/owner queues; SOC determines sign-in/session provenance and containment scope; IAM reviews CA exception and method enforcement; IG assesses Anyone link/permissions and authorizes remediation; Privacy Office determines whether privacy investigation, risk assessment or notifications are indicated. Next checkpoint: restricted task acknowledgment and specialist findings. Case remains open/pending.

## Warm-transfer summaries — required standard format

These are **prepared transfer records, not transmitted messages**. All use the approved restricted ServiceNow security/privacy incident task, evidence references EX-01–EX-05 and minimum-necessary redacted text. Timestamp 11/11/2026 14:00 CST identifies summary preparation; actual transfer and acknowledgment times are pending.

### WT-SOC — Contoso Health Security Operations Center (SOC)

| Routing header | Entry |
|---|---|
| To | Contoso Health Security Operations Center (SOC) |
| From | Tier 1 Healthcare IT Support |
| Ticket | TKT-00517 |
| Transfer timestamp | Prepared 11/11/2026 14:00 CST (15:00 ET); dispatch time pending |
| Approved channel / record ID | Restricted ServiceNow security/privacy incident task; TKT-00517 (subtask ID not assigned in source packet) |
| Urgency | IR-01.B: immediate SOC transfer required by training runbook; ticket source P2 (not a new Tier 1 severity rating). |
| Anonymized incident description | Attending Physician entered credentials on a deceptive support link; potentially risky authentication and ePHI-linked activity require triage. |
| Observed signals / check outcomes | 1. EX-02: 09/03/2026 08:14 ET suspicious support-impersonation message; EX-01: credentials entered ~08:17 ET. 2. EX-03: Phoenix success 08:29 ET with unfamiliar-properties risk, and failure 50053 at 08:36 ET. 3. EX-05: FileAccessed 08:32 ET by affected session. |
| Actions already taken | Ticket source opened 09/03/2026 08:44 ET (assigned Tier 1 queue); source evidence catalogued/redacted into EX-01–EX-05 for this training report. No live account disablement, password reset, session revocation or user notification verified; those actions remain pending authorized responders. |
| Evidence preserved | Source packet unchanged; EX-01–EX-05 redacted digests; controlled original repository placement not independently verified. No unredacted evidence attached. |

| Routing header | Entry |
|---|---|
| Specific action requested | Triage phishing / Entra signals and session provenance, coordinate clinically safe containment with IAM, correlate FileAccessed and other authorized records, advise next actions under IR-01.B and FLR-01. |
| Unresolved question | Was the successful Phoenix sign-in associated with an unauthorized actor or active compromised session? |
| Acknowledgment / new owner | Pending: no real ServiceNow dispatch, acknowledgment timestamp, or receiving owner verified. |
| Pre-send redaction check | Clean — provider/workforce/patient identifiers, email addresses, PHI-bearing filename/path, subject, attachment names and hyperlinks omitted or substituted. |

## WT-IAM — Identity & Access Management Operations

| Routing header | Entry |
|---|---|
| To | Identity & Access Management Operations |
| From | Tier 1 Healthcare IT Support |
| Ticket | TKT-00517 |
| Transfer timestamp | Prepared 11/11/2026 14:00 CST (15:00 ET); dispatch time pending |
| Approved channel / record ID | Restricted ServiceNow security/privacy incident task; TKT-00517 (subtask ID not assigned in source packet) |
| Urgency | IR-01.B; policy configuration remediation indicated; authority and care-continuity checks required. |
| Anonymized incident description | Affected Workforce Account has a clinical Conditional Access policy gap in the setting of reported credential entry and suspicious sign-in activity. |
| Observed signals / check outcomes | 1. EX-04: CA-CLINICAL-MFA did not apply; CA-TEMP-EXCEPTION retained without an end date (snapshot timestamp not provided). 2. EX-04: Authenticator registered, no FIDO2 registration; registered ≠ enforced. 3. EX-03: 08:29 ET risky Phoenix success; 08:36 ET risk-blocked failure. |
| Actions already taken | Ticket source opened 09/03/2026 08:44 ET (assigned Tier 1 queue); source evidence catalogued/redacted into EX-01–EX-05 for this training report. Only documentation of observed state. No Conditional Access, MFA, group membership or account changes performed. |
| Evidence preserved | Source packet unchanged; EX-01–EX-05 redacted digests; controlled original repository placement not independently verified. No unredacted evidence attached. |
| Specific action requested | Review exception legitimacy and approval, repair policy drift if authorized; assess per-sign-in MFA enforcement; coordinate user/session containment and care continuity with SOC; document before/after status and assigned owner. |
| Unresolved question | Was exception authorized and current? Was MFA required/satisfied on 08:29 ET sign-in, and which remediation is approved? |
| Acknowledgment / new owner | Pending: no real ServiceNow dispatch, acknowledgment timestamp, or receiving owner verified. |
| Pre-send redaction check | Clean — provider/workforce/patient identifiers, email addresses, PHI-bearing filename/path, subject, attachment names and hyperlinks omitted or substituted. |

## WT-IG — Information Governance

| Routing header | Entry |
|---|---|
| To | Information Governance |
| From | Tier 1 Healthcare IT Support |
| Ticket | TKT-00517 |
| Transfer timestamp | Prepared 11/11/2026 14:00 CST (15:00 ET); dispatch time pending |

| Routing header | Entry |
|---|---|
| Approved channel / record ID | Restricted ServiceNow security/privacy incident task; TKT-00517 (subtask ID not assigned in source packet) |
| Urgency | M365-01 / PRIV-01.B governance review; no Tier 1 severity assessment. |
| Anonymized incident description | Shared emergency-department clinical case artifact containing possible ePHI has a broad anonymous link and a related audit event. |
| Observed signals / check outcomes | 1. EX-05: Artifact A-01 stored in SharePoint; service encryption active. 2. EX-05: Anyone link created 08/28/2026; no expiration or password. 3. EX-05: FileAccessed 09/03/2026 08:32 ET by affected session; direct/group/guest permissions not supplied. |
| Actions already taken | Ticket source opened 09/03/2026 08:44 ET (assigned Tier 1 queue); source evidence catalogued/redacted into EX-01–EX-05 for this training report. Redacted configuration and event findings documented; no link revocation, permissions changes, or file edits executed. |
| Evidence preserved | Source packet unchanged; EX-01–EX-05 redacted digests; controlled original repository placement not independently verified. No unredacted evidence attached. |
| Specific action requested | Preserve current link/permission/guest and audit details before approved change; check site baseline, permission inheritance, recipients, retention and scope; authorize and record corrective sharing configuration if needed; coordinate SOC/Privacy findings. |
| Unresolved question | Who could use the link, was the link used, and which governance remediation is required? |
| Acknowledgment / new owner | Pending: no real ServiceNow dispatch, acknowledgment timestamp, or receiving owner verified. |
| Pre-send redaction check | Clean — provider/workforce/patient identifiers, email addresses, PHI-bearing filename/path, subject, attachment names and hyperlinks omitted or substituted. |

## WT-PRIV — HIPAA Privacy & Compliance Office

| Routing header | Entry |
|---|---|
| To | HIPAA Privacy & Compliance Office |
| From | Tier 1 Healthcare IT Support |
| Ticket | TKT-00517 |
| Transfer timestamp | Prepared 11/11/2026 14:00 CST (15:00 ET); dispatch time pending |
| Approved channel / record ID | Restricted ServiceNow security/privacy incident task; TKT-00517 (subtask ID not assigned in source packet) |
| Urgency | PRIV-01.A and PRIV-01.B: privacy review triggered in parallel; determination reserved under PRIV-02. |
| Anonymized incident description | Possible ePHI linked to an account with suspected credential/session misuse and an over-broad clinical sharing link; extent and actor are unresolved. |
| Observed signals / check outcomes | 1. Component 1 EX-01/02: credential entry ~08:17 ET; EX-03 Phoenix risk signal at 08:29 ET. 2. Component 2 EX-04: CA exception lacks end date, clinical MFA policy not applied. 3. Component 3 EX-05: Anyone link from 08/28 and FileAccessed 09/03/2026 08:32 ET. 4. Component 4: redacted digests EX-01–EX-05 reviewed clean. 5. HIPAA checklist A–D: security record opened, ePHI possible, access unconfirmed, privacy determinations unresolved, handoff ownership pending. |
| Actions already taken | Ticket source opened 09/03/2026 08:44 ET (assigned Tier 1 queue); source evidence catalogued/redacted into EX-01–EX-05 for this training report. Runbook fit evaluated under IR-01.B/C, PRIV-01.A/B, EVID-01, M365-01 and PRIV-02; no unauthorized-access, impermissible-use, breach, severity or notification judgment made. |
| Evidence preserved | Source packet unchanged; EX-01–EX-05 redacted digests; controlled original repository placement not independently verified. No unredacted evidence attached. |
| Specific action requested | Assess privacy implications; determine whether impermissible use/disclosure, applicable exceptions, four-factor assessment and notifications require evaluation; coordinate with SOC/IAM/IG using restricted evidence and mitigation records. |

| Routing header | Entry |
|---|---|
| Unresolved question | Did any unauthorized acquisition or viewing of Patient A clinical information occur, and what privacy process/notification (if any) is required? |
| Acknowledgment / new owner | Pending: no real ServiceNow dispatch, acknowledgment timestamp, or receiving owner verified. |
| Pre-send redaction check | Clean — provider/workforce/patient identifiers, email addresses, PHI-bearing filename/path, subject, attachment names and hyperlinks omitted or substituted. |

## Final submission and integrity checklist

• Six required components and case summary present in specified order.

• Four complete warm-transfer summaries with correct receiving functions and evidence references.

• No original PHI, identifying email, provider name, MRN, DOB, or PHI-bearing filename reproduced.

• No autonomous breach, actual-access, risk classification, or notification conclusion.

• Prepared versus completed actions and acknowledgment statuses distinguished; no invented production interventions.

• Source evidence unchanged; final PDF text and metadata verified on export.